

Guide 54 — Occupational Therapy Assistant

Controlled English working master — Version 2.0

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What this work is

An occupational therapy assistant (OTA) helps occupational therapists deliver occupational therapy services to clients who are developing, recovering, maintaining, or adapting the skills needed for daily life. In the United States, an OTA may work with people of many ages in rehabilitation clinics, hospitals, nursing facilities, schools, home-health settings, community programs, and other environments where occupational therapy is provided.

An OTA is not the same as an occupational therapist, and an OTA is not the same as an occupational therapy aide. The occupational therapist evaluates the client, establishes the occupational-therapy plan and goals, and performs other functions reserved by law, regulation, professional standards, and employer policy. The OTA contributes to implementation, observation, documentation, education, and feedback within the occupational-therapy process and within the supervision and scope requirements that apply to the setting and jurisdiction.

Occupational therapy aides generally perform routine support tasks and do not follow the same accredited education, national certification, or state regulatory pathway as occupational therapy assistants.

This guide is for career exploration and preparation. Education, certification, licensure, supervision, scope, funding, and employment rules can change. Verify the requirements that apply where you intend to study and work before paying tuition, accepting a job, or performing clinical tasks.

What an occupational therapy assistant may do

Typical OTA responsibilities can include:

- carrying out therapeutic activities and interventions that are part of the occupational therapist's established plan;
- helping clients practice activities of daily living such as dressing, grooming, meal preparation, household tasks, school tasks, work-related activities, or community participation;
- guiding therapeutic exercises, functional activities, play, or task practice within the approved plan and the OTA's competence;

- teaching clients how to use adaptive equipment or strategies that have been selected within the occupational-therapy process;
- observing how clients respond to treatment and reporting progress, barriers, pain, fatigue, behavior changes, or safety concerns;
- documenting services and client responses accurately and on time;
- helping prepare treatment materials, adaptive devices, activity stations, or the clinical environment;
- collaborating with occupational therapists and other members of the care or education team;
- reinforcing safety, home-program, caregiver, or classroom instructions that are within the established plan and the assistant's scope; and
- escalating questions that require evaluation, a plan change, diagnosis, discharge judgment, or another regulated professional decision.

Exact duties depend on state law, the occupational therapist's plan and supervision, the clinical or educational setting, payer rules, employer policy, and the client's condition.

What an OTA must not assume

Do not assume that OTA education permits you to:

- independently diagnose a medical, developmental, psychiatric, neurological, or occupational-performance condition;
- independently perform the occupational therapist's evaluation or establish the initial occupational-therapy plan;
- independently set or change goals outside the authority allowed by law and supervision;
- independently discharge a client when that decision is reserved to the occupational therapist;
- perform an intervention, transfer, modality, splinting task, feeding activity, cognitive intervention, or other procedure for which you have not been trained and authorized;
- ignore required supervision or documentation rules;
- give medical, legal, financial, or mental-health advice outside your professional role; or
- present a foreign or assistant-level credential as equivalent to another jurisdiction's regulated occupational therapist title.

When scope is unclear, stop and verify with the supervising occupational therapist, employer, and applicable regulator.

Work settings and physical demands

Occupational therapy assistants may work in:

- outpatient occupational-therapy or rehabilitation practices;
- hospitals;
- skilled nursing and long-term care facilities;
- home healthcare;
- schools and educational programs;
- pediatric therapy programs;
- mental-health or community settings;
- rehabilitation centers;
- disability-support programs; and
- other settings where occupational therapy services are delivered.

The work can involve standing, walking, bending, kneeling, reaching, handling equipment, assisting with transfers, guarding clients during mobility or functional tasks, and adapting the environment. Some roles include frequent documentation, computer use, driving between clients, or carrying treatment materials.

A client's physical, cognitive, emotional, sensory, or behavioral condition can change unexpectedly. Follow approved patient-handling techniques and request help when a transfer, behavior, feeding task, equipment setup, or other activity exceeds your competence or safe physical limits.

Schedules vary. Hospitals, skilled nursing, home health, and some outpatient or pediatric settings may require evenings, weekends, travel, variable caseloads, or productivity expectations.

Skills employers value

Strong occupational therapy assistants typically develop:

- close observation and accurate reporting;
- patient, calm communication;
- practical teaching and demonstration skills;
- safe patient handling and positioning;
- empathy without overstepping professional boundaries;
- accurate documentation;
- teamwork with occupational therapists, nurses, physicians, teachers, speech-language pathologists, physical therapists, social workers, caregivers, and other professionals;
- infection-prevention awareness;
- time and caseload management;
- basic digital literacy for electronic health records, scheduling, telehealth-support systems, or education systems when applicable;
- cultural humility and respect for client goals and routines;

- awareness of sensory, cognitive, physical, emotional, and environmental barriers; and
- judgment about when to stop and escalate.

The work also benefits from creativity. Occupational therapy often uses real-life tasks and meaningful activities, but creativity must remain within the occupational therapist's plan, the client's goals, the evidence base, and the assistant's authorized scope.

Education and entry pathway — United States

The U.S. Bureau of Labor Statistics states that occupational therapy assistants typically need an associate degree from an accredited occupational therapy assistant program and usually need a state-issued license.

The Accreditation Council for Occupational Therapy Education (ACOTE) accredits entry-level occupational therapy and occupational therapy assistant educational programs. ACOTE states that entry into the OTA profession may occur at the associate or bachelor degree level. Program status can change, so verify the exact school and exact OTA program in ACOTE's live directory before enrollment.

A practical U.S. sequence is:

1. Review local OTA job postings and the occupational-therapy licensing or regulatory board for the state where you intend to work.
2. Identify ACOTE-accredited OTA programs and confirm each program's current accreditation status.
3. Compare prerequisites, tuition, fees, completion requirements, fieldwork structure, travel expectations, graduation rates, certification-exam preparation, and total cost.
4. Apply for grants, scholarships, federal student aid, workforce funding, and employer assistance before relying on private loans.
5. Complete the required academic education and fieldwork.
6. Meet NBCOT eligibility requirements and apply for the COTA certification examination.
7. Complete the state licensing or regulatory process required in the jurisdiction where you intend to practice.
8. Maintain certification, license, continuing education, background checks, CPR/BLS, or other requirements that apply to your jurisdiction and employer.

Graduation, national certification, and state licensure are related but distinct steps. Do not assume that completing one step automatically completes the others.

Accreditation and program verification

ACOTE is the specialized accrediting agency for occupational therapy and occupational therapy assistant educational programs. Its current directory identifies programs and their accreditation status.

Before enrolling, confirm:

- the exact program's current ACOTE status;
- whether the program is associate-level or bachelor-level;
- the credential awarded;
- whether the program satisfies the education pathway for the jurisdiction where you plan to seek licensure;
- tuition and mandatory institutional fees;
- books, uniforms, lab supplies, background checks, drug screens, immunizations, CPR/BLS, technology, transportation, and fieldwork costs;
- whether fieldwork placements can require travel or schedule changes;
- whether the program publishes graduation and NBCOT examination outcomes;
- disability-access and testing-accommodation procedures;
- refund and withdrawal policies; and
- whether credits transfer if you later pursue additional education.

Do not rely only on a school's advertising language. A program can use words such as occupational therapy, rehabilitation, therapy assistant, health sciences, or allied health without being an ACOTE-accredited OTA program.

NBCOT certification and state licensure — United States

The National Board for Certification in Occupational Therapy (NBCOT) administers the national certification examination for occupational therapy professionals. For a U.S.-educated COTA candidate, NBCOT states that the candidate must graduate from an ACOTE-accredited occupational therapy assistant program.

Passing the NBCOT examination and meeting certification requirements leads to the **Certified Occupational Therapy Assistant (COTA®)** credential. Certification does not replace state licensure. NBCOT directs candidates to meet all requirements of the jurisdiction where they intend to practice.

NBCOT describes the OTR/COTA professional relationship as collaborative. The occupational therapist evaluates the client, plans therapeutic interventions, and completes discharge documents. The COTA implements and documents therapeutic interventions and gives the occupational therapist feedback about the client's progress.

Before scheduling the certification exam, review NBCOT's current examination handbook for:

- eligibility;
- application procedures;
- current examination fees;
- testing accommodations;
- identification requirements;
- scheduling and testing-center rules;
- rescheduling or cancellation rules;
- retake rules; and
- score reporting.

Then verify state-specific requirements through the applicable state board. Requirements may include application fees, background checks, jurisprudence requirements, temporary permits, continuing education, supervision rules, and renewal procedures.

Income and outlook — United States official data

The U.S. Bureau of Labor Statistics reports the following for occupational therapy assistants:

- **Median annual wage, May 2024: USD \$68,340**
- **Employment in 2024: about 49,200 OTA jobs**
- **Projected OTA employment in 2034: about 58,700 jobs**
- **Projected OTA employment growth, 2024-2034: 19%**

BLS reports these May 2024 median annual wages in major OTA industries:

- Home healthcare services: **\$76,800**
- Nursing care facilities: **\$75,860**
- Offices of physical, occupational and speech therapists and audiologists: **\$65,590**
- Hospitals: **\$65,280**
- Educational services: **\$59,240**

These are national statistics, not promised starting salaries. Actual pay varies by location, experience, employer, schedule, hours worked, setting, union status, reimbursement environment, and local labor demand.

Current private salary estimate — keep separate from official data

Salary.com reported an average U.S. Occupational Therapy Assistant salary of approximately **USD \$62,501 per year / \$30 per hour as of August 1,**

2026, with a stated 25th–75th percentile range of approximately **\$56,882–\$68,464**.

This is a commercial market estimate, not government wage data. Keep it separate from the BLS median and do not present it as a guaranteed salary.

Canada — related title, broader occupational group

Canada's Job Bank maps **occupational therapy assistant** within **NOC 32109 — Other technical occupations in therapy and assessment**. The NOC group includes several technical therapy and assessment occupations and may include workers who assist occupational therapists, physiotherapists, speech-language pathologists, audiologists, and other professionals.

Job Bank describes the broad group as usually requiring college or apprenticeship-level preparation, but that group-level summary is not a single national credential rule for every occupational therapy assistant job.

Verify:

- the exact employer's education requirements;
- the province or territory;
- whether the job is an OTA-only role, combined OT/PT assistant role, rehabilitation-assistant role, or another NOC 32109 occupation;
- duties and supervision;
- whether a provincial regulator or employer imposes additional requirements;
- language requirements; and
- work authorization.

A U.S. COTA credential does not automatically create Canadian recognition, licensure, registration, or employment eligibility.

Canada wage evidence

Government of Canada Job Bank reports national hourly wages for occupational therapy assistant / NOC 32109 as:

- **Low: CAD \$17.62/hour**
- **Median: CAD \$26.85/hour**
- **High: CAD \$36.71/hour**

Job Bank states that these wages were updated November 19, 2025 and use a 2023–2024 Labour Force Survey reference period. Job Bank also reports that **78.8%** of workers in the NOC 32109 group receive at least one non-wage benefit.

Prospects differ by region. Job Bank reports a **Good** outlook for NOC 32109 in Ontario for 2025–2027, but that should not be generalized to every province, city, or exact job title.

Colombia — do not translate a U.S. OTA into the regulated professional title

Colombia regulates **Terapia Ocupacional** as a university-level profession under **Ley 949 de 2005**.

The law defines occupational therapy as a university-trained liberal profession and assigns professional responsibilities to the *Terapeuta Ocupacional*. Article 22 states that occupational-therapy assessment, diagnosis, or action planning involving service users cannot be delegated by the occupational therapist to technicians, technologists, or other persons who are not competent under the law. Article 35 establishes title and professional-card requirements for lawful practice as a *Terapeuta Ocupacional* in Colombia, including convalidation where applicable.

Therefore:

- do not translate a U.S. **Occupational Therapy Assistant, OTA**, or **COTA** as if the person were a Colombian **Terapeuta Ocupacional**;
- do not assume that a U.S. assistant credential authorizes the Colombian professional scope;
- do not assume that a Colombian technical, technological, caregiver, rehabilitation-support, or health-support program is equivalent to a U.S. ACOTE-accredited OTA program; and
- verify Colombian education and professional requirements with competent authorities before making enrollment or practice decisions.

People exploring rehabilitation-related work in Colombia should distinguish among the regulated university profession, technical or auxiliary support work, disability-support roles, health-services support, education support, and employer-specific rehabilitation roles.

Colombia and Latin America public-training exploration

SENA and the Colombian Ministry of Education are useful starting points for public and recognized education exploration. Program availability changes by region and intake.

SENA may offer adjacent training in areas such as health services, disability support, caregiving, physical activity, occupational safety, community services, or other practical fields. These can be useful careers in their own right, but they should not be marketed as the Colombian regulated profession of occupational therapy unless the competent authority confirms that status.

Across Latin America, titles such as occupational therapist, therapy assistant, rehabilitation technician, rehabilitation aide, and health technician do not have uniform legal meaning. Use national ministries, public training services, professional regulators, universities, and recognized vocational institutions to verify each country separately.

Free and low-cost funding — United States

WIOA and American Job Centers

The Workforce Innovation and Opportunity Act (WIOA) can support eligible occupational skills training through state and local workforce systems. The U.S. Department of Labor maintains continuing guidance for Eligible Training Providers and programs.

If you may qualify:

1. Contact an American Job Center.
2. Ask whether the exact OTA program appears on the applicable state Eligible Training Provider list.
3. Confirm your personal eligibility and the amount/types of assistance available.
4. Ask what costs can be covered, such as tuition, books, transportation, uniforms, testing, or supportive services.
5. Get approval before paying costs that you expect WIOA to reimburse.

An eligible program does not guarantee that a particular student will receive funding.

Federal Student Aid

Use **StudentAid.gov** to complete the FAFSA and verify current federal aid rules. Depending on eligibility and the participating institution/program, aid may include grants, work-study, and federal student loans.

Before borrowing:

- compare grants and scholarships first;
- calculate the full program cost;
- estimate travel and fieldwork costs;
- compare federal loans with private loans;
- understand interest and repayment terms; and
- avoid assuming that a future OTA salary will automatically make any debt affordable.

Employer and school assistance

Ask hospitals, rehabilitation systems, skilled nursing organizations, home-health providers, schools, and large therapy practices about:

- tuition reimbursement;
- scholarships;
- paid training;
- CPR/BLS reimbursement;
- fieldwork partnerships;
- continuing-education assistance;
- internal advancement from aide/support roles; and
- service commitments or repayment clauses.

Get repayment obligations in writing before accepting employer education money.

Funding — Canada

Government of Canada Student Aid and provincial or territorial student-aid programs can help eligible learners with postsecondary costs. Designated-school and program rules matter.

Before enrolling, verify:

- that the school/program is eligible for the aid you plan to use;
- provincial or territorial application procedures;
- grant and loan amounts;
- residency and immigration-status requirements;
- disability-related supports; and
- any current restrictions affecting private or for-profit schools.

Also ask colleges about bursaries, mature-student aid, emergency grants, accessibility funding, and employer partnerships.

Apprenticeship and work-based learning

Do not advertise the U.S. OTA pathway as a shortcut apprenticeship. The standard U.S. pathway is an ACOTE-accredited occupational therapy assistant education program that includes required fieldwork.

Fieldwork is authentic supervised professional learning, but it is not automatically a Registered Apprenticeship and should not be described as one unless the program is formally registered as such.

Canada's broad NOC 32109 may include occupations with different preparation routes. Verify the exact program and job rather than assuming that all Canadian OTA roles share one apprenticeship structure.

A practical 12-week exploration plan

Weeks 1-2: Read 15–20 local OTA postings. Record repeated requirements: license, COTA, CPR/BLS, schedule, setting, experience, documentation systems, lifting/transfer expectations, pediatrics or geriatrics, and travel.

Weeks 3-4: Identify ACOTE-accredited OTA programs within a realistic commuting or relocation range. Compare associate and bachelor options when relevant. Record total cost, prerequisites, fieldwork structure, completion outcomes, and current accreditation status.

Weeks 5-6: Refresh anatomy, medical terminology, human development, basic psychology, communication, infection prevention, documentation concepts, and safe body mechanics with free or low-cost resources. Do not practice clinical procedures on patients without authorized instruction and supervision.

Weeks 7-8: Complete FAFSA and investigate WIOA, scholarships, school grants, employer assistance, and local foundations. Build a written cost comparison for at least three options where possible.

Weeks 9-10: Speak with admissions staff and practicing occupational therapy professionals if available. Ask about fieldwork travel, evening/weekend requirements, background checks, immunizations, CPR/BLS, technology, licensure preparation, and physical demands.

Weeks 11-12: Confirm NBCOT and state licensure requirements. Decide whether the occupation's client contact, emotional demands, physical demands, education cost, schedule, and local compensation fit your goals and circumstances.

Building experience before becoming an OTA

Depending on local law and employer policy, useful exposure may come from non-OTA roles such as:

- occupational therapy aide;
- rehabilitation aide or rehabilitation technician;
- patient-care support;
- nursing assistant;
- caregiver or disability-support worker;
- activity assistant in senior care;
- school classroom support;
- behavioral-health support roles;
- healthcare receptionist or scheduler; or
- supervised volunteer work in disability, aging, pediatric, or rehabilitation programs.

Do not describe support work as licensed or certified OTA experience if it was not.

Career progression

An OTA can build depth in settings such as geriatrics, pediatrics, schools, physical rehabilitation, neurological rehabilitation, mental health, home health, skilled nursing, hand therapy support, or community practice when permitted and properly supervised.

Experienced OTAs may also move toward:

- lead or coordination responsibilities;
- fieldwork education support;
- rehabilitation operations;
- quality or documentation support;
- assistive-technology support;
- disability-services roles;
- healthcare training; or
- further education.

Becoming an occupational therapist requires a separate professional education and credential pathway. OTA experience is valuable, but it does not itself convert into occupational therapist licensure.

Client safety and escalation

Client safety takes priority over productivity, schedule, or convenience. Follow the occupational therapist's plan, supervision requirements, facility procedures, and your training.

Stop and escalate according to policy for concerns such as:

- chest pain, severe shortness of breath, fainting, sudden weakness, or signs of a medical emergency;
- a fall or near-fall;
- new severe pain;
- significant changes in cognition, alertness, behavior, or neurological function;
- suicidal or violent statements or other urgent behavioral-health risks;
- suspected abuse, neglect, exploitation, or unsafe living conditions;
- swallowing, choking, feeding, aspiration, or respiratory concerns during an activity;
- skin injury, pressure areas, burns, or device-related injury;
- equipment failure;
- a transfer or task that cannot be completed safely;

- a client request that is outside the plan or your scope; or
- any situation that your training tells you requires the occupational therapist, nurse, physician, emergency services, safeguarding process, or another professional.

Do not improvise outside your competence when safety is uncertain.

Privacy, documentation, and cybersecurity

Occupational therapy assistants may handle health, education, disability, contact, insurance, scheduling, and other sensitive information.

Use only approved systems and follow the privacy rules that apply to your organization and jurisdiction. Good habits include:

- confirming identity before discussing protected information;
- using only approved electronic health record, education, messaging, telehealth, and storage systems;
- locking screens when unattended;
- using strong unique passwords and multifactor authentication when available;
- refusing unexpected password-reset, payment, gift-card, or remote-access requests;
- checking recipients before sending records;
- avoiding personal cloud storage, personal email, or unapproved messaging for client records;
- documenting objectively and promptly;
- reporting lost devices, suspected phishing, misdirected records, or privacy incidents immediately; and
- discussing clients only with authorized people who need the information.

Do not copy client information into public AI systems, personal note applications, or unapproved tools.

Responsible AI in occupational-therapy support work

AI can help with low-risk administrative or learning tasks when employer policy permits, but it does not replace clinical judgment, the occupational therapist's evaluation, or professional accountability.

Possible low-risk uses include:

- creating generic study questions that contain no client information;
- simplifying general educational language for review by the appropriate professional;

- brainstorming non-client-specific activity ideas for later clinical review;
- drafting generic interview practice questions;
- organizing a personal study schedule; or
- checking grammar in non-confidential material.

Do not use public or unapproved AI tools to:

- enter identifiable client information;
- generate or alter clinical facts;
- diagnose a client;
- independently select or change treatment goals;
- fabricate documentation;
- invent measurements or observations;
- hide errors;
- make discharge decisions; or
- override the occupational therapist, employer policy, or applicable law.

Treat AI output as a draft that can be wrong. Verify facts, preserve privacy, and follow your organization's policy.

Scam detection and spending-pause checks

Healthcare education is expensive enough that every major payment deserves verification.

Pause before paying when a school, recruiter, credential service, or training provider:

- guarantees licensure, certification, employment, immigration status, or a specific salary;
- pressures you to pay immediately;
- claims “national accreditation” without naming the recognized accreditor;
- says the program is “ACOTE aligned” but is not listed with the required current status;
- asks for tuition through cryptocurrency, gift cards, wire transfers to individuals, or unusual payment apps;
- promises you can become an occupational therapist through a very short assistant course;
- offers a fake COTA credential without NBCOT eligibility and examination requirements;
- asks you to buy equipment or send money to receive a job; or
- refuses to provide written tuition, refund, accreditation, or credential information.

Before a major payment, independently verify the school, program accreditation, licensing pathway, total cost, refund policy, and financial-aid terms.

Choosing a program — decision checklist

A strong program decision should answer:

- Is the exact OTA program currently ACOTE accredited or in an accreditation status that supports the pathway I intend to use?
- What credential will I earn?
- Does the program satisfy the education requirements of the jurisdiction where I plan to seek licensure?
- What is the total cost after grants and scholarships?
- How much debt would I need?
- How far can fieldwork placements be from campus or home?
- Can I meet daytime, evening, weekend, or travel requirements?
- What are the physical and health-document requirements?
- What disability accommodations are available?
- What are the completion and NBCOT examination outcomes?
- What is the refund/withdrawal policy?
- What do local employers actually require?
- Is local pay reasonable relative to tuition, debt, transportation, and living costs?

If these answers are unclear, pause before paying.

Questions to ask an employer

Useful interview questions include:

- How is OTA supervision structured here?
- What is the typical caseload and productivity expectation?
- Which client populations and diagnoses are most common?
- How much documentation time is built into the schedule?
- Are evenings, weekends, holidays, or travel required?
- What lifting, transfer, feeding, splinting, or equipment competencies are expected?
- What orientation and competency validation are provided?
- How are urgent safety concerns escalated?
- Which electronic health record or documentation system is used?
- What continuing education or tuition assistance is available?
- How does the organization protect time for collaboration between the occupational therapist and OTA?

Portfolio and job-search preparation

A new OTA portfolio can include non-confidential evidence such as:

- résumé and targeted cover letter;
- current license or permit details where appropriate;
- COTA certification status;
- CPR/BLS documentation if required;
- fieldwork setting summaries that reveal no client identity;
- school projects stripped of protected information;
- continuing-education certificates;
- assistive-technology or adaptive-equipment training;
- quality-improvement or teamwork examples; and
- references who have agreed to be contacted.

Never place protected client information, identifiable photographs, medical records, education records, or employer-confidential material in a portfolio.

Résumé language that stays accurate

Use evidence-based statements such as:

- “Implemented occupational-therapy interventions under the supervision and plan of the occupational therapist.”
- “Documented client response and communicated progress and safety concerns to the supervising occupational therapist.”
- “Supported activities of daily living, adaptive-equipment training, and functional task practice within established plans.”
- “Completed fieldwork in pediatric, rehabilitation, school, skilled-nursing, or other verified settings.”

Do not claim that you diagnosed clients, independently created plans of care, held a license you do not have, or used a modality or specialty technique that you were not trained to perform.

Accessibility and disability support

Students and workers with disabilities can succeed in occupational therapy assistant pathways, but the combination of clinical fieldwork and essential job functions makes early planning important.

Before enrollment, ask the school about:

- disability-services procedures;
- classroom and laboratory accommodations;
- testing accommodations;
- fieldwork accommodation planning;

- accessible technology and course materials;
- attendance and scheduling requirements;
- safe performance of essential clinical tasks; and
- how accommodation requests are coordinated with fieldwork sites.

NBCOT also publishes testing-accommodation procedures. Apply early and follow the current documentation requirements.

An accommodation does not remove legitimate essential competencies or patient-safety requirements, but schools and employers may have obligations to provide reasonable accommodations under applicable law.

Source list — verify before important decisions

United States

- U.S. Bureau of Labor Statistics — Occupational Therapy Assistants and Aides: [Source link](#)
- ACOTE — accredited programs and accreditation information: <https://acoteonline.org/all-schools/>
- ACOTE — current standards: [Source link](#)
- NBCOT — certification pathway: <https://www.nbcot.org/get-certified>
- NBCOT — eligibility: [Source link](#)
- NBCOT — OTR/COTA role explanation: [Source link](#)
- Federal Student Aid: <https://studentaid.gov/>
- U.S. Department of Labor WIOA guidance: [Source link](#)

Canada

- Government of Canada Job Bank — Occupational Therapy Assistant / NOC 32109: [Source link](#)
- Job Bank wages: [Source link](#)
- Canada Student Aid: [Source link](#)

Colombia / Latin America

- Colombia Ley 949 de 2005 — SUIN-Juriscol: [Source link](#)
- SENA: <https://www.sena.edu.co/>
- Ministerio de Educación Nacional: <https://www.mineducacion.gov.co/>

Private salary supplement

- Salary.com — Occupational Therapy Assistant Salary, updated August 1, 2026: [Source link](#)

Final reality check

Occupational therapy assistant work can provide meaningful direct service, strong projected U.S. demand, and multiple practice settings. It also carries substantial responsibility. You may work with people who are injured, medically fragile, cognitively impaired, emotionally distressed, developing new skills, or adapting to disability.

The safest path is not the fastest advertisement. Verify accreditation, certification eligibility, licensure, scope, fieldwork, total cost, funding, physical demands, and local job conditions before committing.

This guide provides general educational and career-planning information. It does not guarantee admission, graduation, certification, licensure, credential recognition, funding, employment, salary, promotion, immigration status, or any other result. It is not medical, legal, financial, or regulatory advice.

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